



ARIZONA HOUSE OF REPRESENTATIVES FLOOR AMENDMENT EXPLANATION

57th Legislature, 2nd Regular Session

Majority Research Staff

HB 2444: pharmacists; independent testing; treatment

LOPEZ FLOOR AMENDMENT

1. Changes the age of a person that a pharmacist may independently initiate treatment on from 6 years of age to 12.
2. Prohibits an employer from overruling a pharmacist's decision to not independently initiate treatment for the authorized conditions.
3. Requires a pharmacist to maintain professional liability insurance if independently initiating treatment for the authorized conditions.
4. Establishes the Independent Testing and Treatment Advisory Committee under the Arizona State Board of Pharmacy to assist in developing written protocols and outlines membership, compensation and protocol review requirements.
5. Outlines requirements for a pharmacist to initiate treatment for human immunodeficiency virus preexposure prophylaxis and postexposure prophylaxis.
6. Defines *preexposure prophylaxis* and *postexposure prophylaxis*.

LOPEZ SUBSTITUTE FLOOR AMENDMENT
HOUSE OF REPRESENTATIVES AMENDMENTS TO H.B. 2444

I move the following SUBSTITUTE amendment to the HEALTH & HUMAN SERVICES Committee Amendment to
HOUSE BILL 2444 (Reference to printed bill)

Amendment instruction key:

[GREEN UNDERLINING IN BRACKETS] indicates text added to statute or previously enacted session law.
[Green underlining in brackets] indicates text added to new session law or text restoring existing law.
[GREEN STRIKEOUT IN BRACKETS] indicates new text removed from statute or previously enacted session law.
[Green strikeout in brackets] indicates text removed from existing statute, previously enacted session law or new session law.
<<Green carets>> indicate a section added to the bill.
<<Green strikeout in carets>> indicates a section removed from the bill.

1 The bill as proposed to be amended is reprinted as follows:

2 Section 1. Title 32, chapter 18, article 3, Arizona Revised
3 Statutes, is amended by adding [section] [sections] 32-1979.04,
4 [32-1979.05 and 32-1979.06.] to read:

5 32-1979.04. Pharmacists; statewide written protocol;
6 independent testing; treatment; health
7 conditions; notification and insurance
8 requirements

9 A. PURSUANT TO A STATEWIDE WRITTEN PROTOCOL APPROVED BY THE BOARD,
10 A PHARMACIST MAY INDEPENDENTLY ORDER, PERFORM AND INTERPRET TESTS THAT ARE
11 AUTHORIZED BY THE UNITED STATES FOOD AND DRUG ADMINISTRATION AND WAIVED
12 UNDER THE CLINICAL LABORATORY IMPROVEMENT AMENDMENTS OF 1988 (P.L.
13 100-578; 102 STAT. 2903; 42 UNITED STATES CODE SECTION 201). A PHARMACIST
14 MAY INDEPENDENTLY INITIATE TREATMENT TO ELIGIBLE PERSONS WHO ARE AT LEAST
15 [SIX] [TWELVE] YEARS OF AGE OR THE AGE AUTHORIZED BY THE TREATMENT,
16 WHICHEVER AGE IS OLDER, AND WHO HAVE TEST RESULTS THAT INDICATE THE NEED
17 FOR TREATMENT, BY A TEST THAT IS AUTHORIZED BY THE UNITED STATES FOOD AND
18 DRUG ADMINISTRATION AND WAIVED UNDER THE CLINICAL LABORATORY IMPROVEMENT
19 AMENDMENTS OF 1988, FOR ANY OF THE FOLLOWING:

- 20 1. INFLUENZA.
21 2. GROUP A STREPTOCOCCUS PHARYNGITIS.
22 3. SARS-COV-2 OR ANY OTHER CORONAVIRUS RESPIRATORY ILLNESS.
23 [4. HUMAN IMMUNODEFICIENCY VIRUS PREEXPOSURE PROPHYLAXIS OR
24 POSTEXPOSURE PROPHYLAXIS.]

1 ~~[5.]~~ [4.] A CONDITION RELATED TO AN EMERGING OR EXISTING PUBLIC
2 HEALTH THREAT IDENTIFIED BY THE DEPARTMENT OF HEALTH SERVICES FOR WHICH A
3 STATEWIDE STANDING ORDER, RULE OR EXECUTIVE ORDER IS ISSUED.

4 B. WHEN DEVELOPING THE STATEWIDE WRITTEN PROTOCOL, THE BOARD SHALL
5 ADDRESS AT A MINIMUM THE FOLLOWING:

6 1. DOCUMENTATION.

7 2. RECORDS RETENTION.

8 3. REFERRALS.

9 4. PATIENT SCREENING REQUIREMENTS AND OBTAINING RELEVANT MEDICAL
10 HISTORY.

11 5. EXCLUSION CRITERIA.

12 6. TREATMENT INSTRUCTIONS BASED ON THE PATIENT'S AGE AND MEDICAL
13 HISTORY.

14 7. FOLLOW-UP MAINTENANCE AND CARE PLANS.

15 8. ~~[ANY NECESSARY]~~ PHARMACIST TRAINING ~~[OR]~~ [AND] CERTIFICATION
16 REQUIREMENTS.

17 C. A PHARMACIST WHO ORDERS OR CONDUCTS TESTING OR TREATS HEALTH
18 CONDITIONS PURSUANT TO SUBSECTION A OF THIS SECTION SHALL USE ANY TEST
19 THAT MAY GUIDE CLINICAL DECISION-MAKING FOR WHICH A WAIVER HAS BEEN
20 OBTAINED UNDER THE CLINICAL LABORATORY IMPROVEMENT AMENDMENTS OF 1988, OR
21 THE FEDERAL RULES ADOPTED THEREUNDER, OR ANY SCREENING PROCEDURE THAT IS
22 ESTABLISHED BY THE STATEWIDE WRITTEN PROTOCOL.

23 D. A PHARMACIST SHALL USE EVIDENCE-BASED CLINICAL GUIDELINES
24 PUBLISHED BY THE UNITED STATES CENTERS FOR DISEASE CONTROL AND PREVENTION
25 OR THE INFECTIOUS DISEASES SOCIETY OF AMERICA, THE AMERICAN ACADEMY OF
26 PEDIATRICS COMMITTEE ON INFECTIOUS DISEASE OR ANOTHER CLINICALLY
27 RECOGNIZED RECOMMENDATION [THAT ALIGNS WITH STANDARDS OF CARE] IN
28 PROVIDING PATIENT TREATMENT PURSUANT TO SUBSECTION A OF THIS SECTION.

29 E. AN ELIGIBLE PERSON MUST MEET CRITERIA FOR TREATMENT BASED ON THE
30 STATEWIDE WRITTEN PROTOCOL THAT SPECIFIES THE FOLLOWING:

31 1. PATIENT INCLUSION AND EXCLUSION CRITERIA.

32 2. EXPLICIT MEDICAL REFERRAL CRITERIA.

33 F. A PHARMACIST SHALL REFER A PATIENT TO THE PATIENT'S PRIMARY CARE
34 PROVIDER, IF ONE IS IDENTIFIED, OR RECOMMEND FOLLOW UP WITH A PRIMARY CARE
35 PROVIDER, IF THE PATIENT EITHER:

36 1. IS NOT ELIGIBLE FOR TREATMENT PURSUANT TO THIS SECTION AND
37 PRESENTS WITH SYMPTOMS.

38 2. DOES NOT RESPOND TO THE INITIAL TREATMENT PROVIDED PURSUANT TO
39 THIS SECTION.

40 G. A PHARMACIST WHO INITIATES A TREATMENT UNDER THIS SECTION SHALL:

41 1. NOTIFY THE PATIENT'S PRIMARY CARE PROVIDER, IF ONE IS
42 IDENTIFIED, WITHIN SEVENTY-TWO HOURS AFTER INITIATING TREATMENT PURSUANT
43 TO THIS SECTION. THE NOTICE SHALL INCLUDE THE PATIENT'S NAME, THE
44 TREATMENT INITIATED AND THE DATE OF TREATMENT AND MAY BE SUBMITTED BY
45 ENTRY INTO AN ELECTRONIC HEALTH RECORD OR BY TELEPHONE, FAX, MAIL OR
46 EMAIL. THE PHARMACIST SHALL MAKE A REASONABLE EFFORT TO IDENTIFY THE
47 PATIENT'S PRIMARY CARE PROVIDER BY AT LEAST ONE OF THE FOLLOWING METHODS:

1 (a) CHECKING PHARMACY RECORDS.

2 (b) REQUESTING THE INFORMATION FROM THE PATIENT OR, FOR A PATIENT
3 UNDER EIGHTEEN YEARS OF AGE, THE PATIENT'S PARENT OR GUARDIAN.

4 2. MAINTAIN [FOR AT LEAST SEVEN YEARS] A RECORD OF THE RESULTS OF
5 ANY TESTING OR SCREENING FOR WHICH A TREATMENT IS INITIATED PURSUANT TO
6 THIS SECTION, INCLUDING A SUMMARY OF THE VISIT~~[.]~~ ~~[AND]~~ PATIENT ASSESSMENT
7 INFORMATION, ~~[FOR A PERIOD OF SEVEN YEARS]~~ [HISTORY OF ILLNESS,
8 EXAMINATION FINDINGS, VITALS AND PLAN OF CARE].

9 3. NOTIFY THE PATIENT'S PRIMARY CARE PROVIDER, IF ONE IS
10 IDENTIFIED, WITHIN FORTY-EIGHT HOURS AFTER THE OCCURRENCE OF ANY ADVERSE
11 REACTION THAT IS REPORTED TO OR WITNESSED BY THE PHARMACIST AS A RESULT OF
12 THE TREATMENT PROVIDED PURSUANT TO THIS SECTION.

13 4. PROVIDE INFORMATIONAL MATERIALS TO THE PATIENT REQUESTING
14 TREATMENT OR, FOR A PATIENT UNDER EIGHTEEN YEARS OF AGE, TO THE PATIENT'S
15 PARENT OR GUARDIAN ABOUT THE IMPORTANCE OF PEDIATRIC PREVENTIVE HEALTH
16 CARE VISITS AS RECOMMENDED BY THE AMERICAN ACADEMY OF PEDIATRICS.

17 H. A PHARMACIST MAY DELEGATE THE TASK OF PERFORMING A TEST WAIVED
18 BY THE CLINICAL LABORATORY IMPROVEMENT AMENDMENTS OF 1988 TO A LICENSED
19 MEMBER OF THE PHARMACY STAFF WHO IS UNDER THE SUPERVISION OF THE
20 PHARMACIST. A PHARMACIST MAY NOT DELEGATE ANY TASKS THAT INCLUDE CLINICAL
21 JUDGMENT OR TREATMENT AND MAY DELEGATE ONLY ANCILLARY DUTIES AS ALLOWED BY
22 BOARD RULES.

23 I. THIS SECTION DOES NOT REQUIRE A PHARMACIST TO PROVIDE THE
24 SERVICES AUTHORIZED BY THIS SECTION. [PURSUANT TO SECTION 32-1901.01,
25 SUBSECTION A, PARAGRAPH 21, AN EMPLOYER MAY NOT OVERRULE A PHARMACIST'S
26 DECISION TO NOT PROVIDE SERVICES THAT ARE AUTHORIZED BY THIS SECTION.]

27 J. THIS SECTION DOES NOT ESTABLISH A CAUSE OF ACTION AGAINST A
28 PATIENT'S PRIMARY CARE PROVIDER FOR ANY ADVERSE REACTION, COMPLICATION OR
29 NEGATIVE OUTCOME ARISING FROM ANY TREATMENT INITIATED BY A PHARMACIST
30 PURSUANT TO THIS SECTION.

31 K. A PHARMACIST MAY NOT INDEPENDENTLY INITIATE A TREATMENT USING
32 OPIOIDS FOR A PATIENT.

33 L. A PHARMACIST MAY NOT INDEPENDENTLY ORDER A TEST OR SCREENING OR
34 TREAT A MINOR WITHOUT THE WRITTEN CONSENT OF THE MINOR'S PARENT OR
35 GUARDIAN.

36 M. A PHARMACY SHALL ~~[EITHER]~~ DISPLAY A NOTICE ~~[OR]~~ ~~[AND]~~ INCLUDE IN
37 A PATIENT'S CONSENT PAPERWORK THAT THE TESTING AND TREATMENT BEING
38 PERFORMED PURSUANT TO THIS SECTION ARE BEING PERFORMED BY A PHARMACIST
39 WITHOUT CONSULTATION WITH OR OVERSIGHT BY A PHYSICIAN AND THAT THE PATIENT
40 SHOULD CONSULT WITH A PRIMARY CARE PROVIDER IF SYMPTOMS CONTINUE.

41 [N. A PHARMACIST SHALL MAINTAIN PROFESSIONAL LIABILITY INSURANCE IF
42 PERFORMING SERVICES THAT ARE AUTHORIZED UNDER THIS SECTION.]

43 32-1979.05. Independent testing and treatment advisory
44 committee; duties; members

45 [A. THE BOARD SHALL ESTABLISH THE INDEPENDENT TESTING AND TREATMENT
46 ADVISORY COMMITTEE TO ASSIST THE BOARD IN DEVELOPING THIS STATE'S WRITTEN
47 PROTOCOLS RELATING TO PHARMACISTS' INDEPENDENT AUTHORITY TO ORDER TESTING

1 AND INITIATE TREATMENTS PURSUANT TO SECTION 32-1979.04. THE ADVISORY
2 COMMITTEE SHALL ALSO MAKE RECOMMENDATIONS TO THE BOARD REGARDING THE
3 PROTOCOLS REQUIRED PURSUANT TO THAT SECTION.

4 B. THE ADVISORY COMMITTEE SHALL INCLUDE AT LEAST THE FOLLOWING:

5 1. TWO PHARMACISTS WHO ARE LICENSED PURSUANT TO THIS CHAPTER AND
6 WHO ARE APPOINTED BY THE BOARD.

7 2. TWO PHYSICIANS WHO ARE LICENSED PURSUANT TO THIS TITLE, ONE WHO
8 SPECIALIZES IN PRIMARY CARE AND ONE WHO PRACTICES IN AN EMERGENCY MEDICINE
9 SETTING, AND WHO ARE APPOINTED BY THE ARIZONA MEDICAL BOARD OR THE ARIZONA
10 BOARD OF OSTEOPATHIC EXAMINERS IN MEDICINE AND SURGERY. AT LEAST ONE OF
11 THESE MEMBERS MUST HAVE A PATIENT POPULATION THAT IS SUBSTANTIALLY
12 COMPOSED OF CHILDREN AND ADOLESCENTS.

13 3. ONE PERSON WHO REPRESENTS A NONPROFIT PATIENT ADVOCACY
14 ORGANIZATION AND WHO IS APPOINTED BY THE GOVERNOR.

15 4. ONE NURSE PRACTITIONER WHO IS LICENSED PURSUANT TO CHAPTER 15 OF
16 THIS TITLE, WHO SPECIALIZES IN PRIMARY CARE OR EMERGENCY MEDICINE, WHO IS
17 ABLE TO PRESCRIBE MEDICATION AND WHO IS APPOINTED BY THE ARIZONA STATE
18 BOARD OF NURSING.

19 C. ADVISORY COMMITTEE MEMBERS ARE NOT ELIGIBLE FOR COMPENSATION OR
20 REIMBURSEMENT OF EXPENSES.

21 D. THE ADVISORY COMMITTEE SHALL REVIEW THE STATEWIDE PROTOCOLS
22 ANNUALLY AND UPDATE WHEN NECESSARY.]

23 32-1979.06. Human immunodeficiency virus prevention;
24 preexposure prophylaxis; postexposure
25 prophylaxis; rules; definitions

26 [A. IN ADDITION TO THE AUTHORITY PROVIDED IN SECTION 32-1979.04, A
27 PHARMACIST WHO IS LICENSED PURSUANT TO THIS CHAPTER MAY INITIATE AND
28 DISPENSE HUMAN IMMUNODEFICIENCY VIRUS PREEXPOSURE PROPHYLAXIS AND
29 POSTEXPOSURE PROPHYLAXIS PURSUANT TO THE STATEWIDE WRITTEN PROTOCOL
30 ADOPTED BY THE BOARD.

31 B. BEFORE INITIATING A PREEXPOSURE PROPHYLAXIS, A PHARMACIST SHALL:

32 1. CONDUCT OR OBTAIN A CLINICAL LABORATORY IMPROVEMENT
33 AMENDMENTS-WAIVED HUMAN IMMUNODEFICIENCY VIRUS TEST AND CONFIRM A NEGATIVE
34 TEST RESULT.

35 2. ASSESS THE PATIENT FOR CONTRAINDICATIONS, DRUG INTERACTIONS AND
36 CLINICAL ELIGIBILITY CONSISTENT WITH THE CURRENT GUIDELINES OF THE UNITED
37 STATES CENTERS FOR DISEASE CONTROL AND PREVENTION.

38 C. BEFORE INITIATING A POSTEXPOSURE PROPHYLAXIS, A PHARMACIST
39 SHALL:

40 1. DETERMINE THAT THE PATIENT PRESENTS WITHIN SEVENTY-TWO HOURS
41 AFTER A POTENTIAL EXPOSURE TO THE HUMAN IMMUNODEFICIENCY VIRUS.

42 2. CONDUCT OR OBTAIN A HUMAN IMMUNODEFICIENCY VIRUS TEST IF
43 FEASIBLE UNDER THE STATEWIDE WRITTEN PROTOCOL.

44 3. ASSESS THE PATIENT FOR CONTRAINDICATIONS, DRUG INTERACTIONS AND
45 CLINICAL ELIGIBILITY CONSISTENT WITH THE CURRENT GUIDELINES OF THE UNITED
46 STATES CENTERS FOR DISEASE CONTROL AND PREVENTION.

47 D. A PHARMACIST WHO INITIATES THERAPY PURSUANT TO THIS SECTION:

1 1. MAY DISPENSE NOT MORE THAN A THIRTY-DAY SUPPLY OF PREEXPOSURE
2 PROPHYLAXIS.

3 2. MAY DISPENSE A COMPLETE TWENTY-EIGHT-DAY COURSE OF POSTEXPOSURE
4 PROPHYLAXIS.

5 3. SHALL PROVIDE COUNSELING REGARDING MEDICATION ADHERENCE,
6 POTENTIAL SIDE EFFECTS AND PREVENTION MEASURES.

7 4. SHALL PROVIDE THE PATIENT WITH A WRITTEN REFERRAL TO A PRIMARY
8 CARE PROVIDER OR OTHER APPROPRIATE HEALTH CARE PROVIDER FOR FOLLOW-UP
9 CARE.

10 5. SHALL NOTIFY THE PATIENT'S PRIMARY CARE PROVIDER OF THE SERVICES
11 PROVIDED IF THE PATIENT CONSENTS TO THE NOTIFICATION.

12 E. THE BOARD SHALL ADOPT RULES NECESSARY TO IMPLEMENT THIS SECTION.
13 INCLUDING:

14 1. MINIMUM TRAINING REQUIREMENTS FOR PHARMACISTS WHO INITIATE
15 THERAPY PURSUANT TO THIS SECTION.

16 2. DOCUMENTATION AND RECORDS RETENTION REQUIREMENTS.

17 3. STATEWIDE WRITTEN PROTOCOL STANDARDS THAT ARE CONSISTENT WITH
18 THE CURRENT GUIDELINES OF THE UNITED STATES CENTERS FOR DISEASE CONTROL
19 AND PREVENTION OR THE INFECTIOUS DISEASES SOCIETY OF AMERICA OR ANOTHER
20 CLINICALLY RECOGNIZED RECOMMENDATION.

21 F. FOR THE PURPOSES OF THIS SECTION:

22 1. "POSTEXPOSURE PROPHYLAXIS" MEANS A MEDICATION REGIMEN THAT IS
23 APPROVED BY THE UNITED STATES FOOD AND DRUG ADMINISTRATION AND THAT IS
24 INITIATED AFTER POTENTIAL EXPOSURE TO THE HUMAN IMMUNODEFICIENCY VIRUS TO
25 PREVENT INFECTION.

26 2. "PREEXPOSURE PROPHYLAXIS" MEANS A MEDICATION REGIMEN THAT IS
27 APPROVED BY THE UNITED STATES FOOD AND DRUG ADMINISTRATION TO PREVENT
28 HUMAN IMMUNODEFICIENCY VIRUS INFECTION IN PERSONS WHO ARE AT RISK.]

29 ~~<<Sec. 2. Independent testing and treatment advisory~~
30 ~~committee; duties; members; delayed repeat~~

31 ~~A. The Arizona state board of pharmacy shall convene an advisory~~
32 ~~committee to assist the board in developing this state's written protocols~~
33 ~~relating to pharmacists' independent authority to order testing and~~
34 ~~initiate treatments pursuant to section 32-1979.04, Arizona Revised~~
35 ~~Statutes, as added by this act. The advisory committee shall also make~~
36 ~~recommendations to the Arizona state board of pharmacy regarding the~~
37 ~~protocols required pursuant to that section.~~

38 ~~B. The advisory committee shall include at least the following:~~

39 ~~1. Two pharmacists who are licensed pursuant to title 32, chapter~~
40 ~~18, Arizona Revised Statutes, and who are appointed by the Arizona state~~
41 ~~board of pharmacy.~~

42 ~~2. Two physicians who are licensed pursuant to title 32, chapter 13~~
43 ~~or 17, Arizona Revised Statutes, who specialize in primary care, at least~~
44 ~~one of whom has a patient population that is substantially composed of~~
45 ~~children and adolescents, and who are appointed by the Arizona medical~~
46 ~~board or the Arizona board of osteopathic examiners in medicine and~~
47 ~~surgery.~~

1 ~~3. One person who represents a nonprofit patient advocacy~~
2 ~~organization and who is appointed by the governor.~~
3 ~~4. One nurse practitioner who is licensed pursuant to title 32,~~
4 ~~chapter 15, Arizona Revised Statutes, who specializes in primary care and~~
5 ~~who is able to prescribe medication and who is appointed by the Arizona~~
6 ~~state board of nursing.~~
7 ~~6. Advisory committee members are not eligible for compensation or~~
8 ~~reimbursement of expenses.~~
9 ~~8. This section is repealed from and after December 31, 2027.>>~~

10 Enroll and engross to conform

11 Amend title to conform

CHRIS LOPEZ

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02/26/2026

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